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REVIEW

Meaning-Centered Psychotherapy in Palliative Care

Conception, State of the Evidence, and Questions of Transferability

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ABSTRACT

In addition to physical symptoms, people with advanced, incurable illnesses frequently face existential distress, such as hopelessness, loss of meaning, or a desire for hastened death. Meaning-centered psychotherapy (MCP), developed by William Breitbart and colleagues, is a manualized intervention that explicitly builds on Viktor E. Frankl's logotherapy and aims to help patients experience meaning even in the face of limited life expectancy. This narrative review first describes the theoretical foundations and the structure of the intervention in its group and individual formats. It then summarizes the evidence from randomized controlled trials, which overall points to favorable effects on spiritual well-being, sense of meaning, and quality of life. Limitations of the existing research are discussed, in particular the concentration on specific populations and health care systems, the handling of attrition in a severely ill population, and questions of cultural transferability. Finally, MCP is compared with other existentially oriented interventions in palliative care, such as dignity therapy. The paper concludes with considerations regarding implementation in German-speaking countries and the qualifications required of clinicians.

Keywords: meaning-centered psychotherapy · palliative care · psycho-oncology · existential distress · logotherapy

1 Introduction

Modern palliative care understands itself as comprehensive support for seriously ill people that takes equal account of the physical, psychological, social, and spiritual dimensions of suffering. Existential burdens at the end of life in particular – the experience of meaninglessness, hopelessness, or the desire for hastened death – pose special challenges for those providing treatment, because they cannot be answered by symptom-oriented measures alone. Yalom (1980) described confronting one's own finitude as a central ultimate concern of existence, which becomes inescapable when facing a life-limiting illness.

Against this background, Meaning-Centered Psychotherapy (MCP) was developed at Memorial Sloan Kettering Cancer Center in New York. It is among the few meaning-centered interventions that have been investigated in several randomized controlled trials and thus represents an important point of reference for the discussion of the evidence base of logotherapeutically inspired procedures. The present contribution provides a narrative overview of its conception and the state of research and asks about its transferability to other care contexts.

Conceptually, existential distress can be distinguished from depressiveness, even though the two may overlap. Hopelessness, loss of meaning, or the feeling of being a burden to others can occur in seriously ill people even without the full picture of a depressive disorder. This gives rise to a need for interventions that explicitly make existential themes their subject.

2 Theoretical Foundations

MCP explicitly draws on the work of Viktor E. Frankl. Its central assumptions are that the striving for meaning represents a fundamental human motivation, that meaning can be experienced even under conditions of severe suffering and limited lifetime, and that people retain the freedom to choose their stance toward unalterable circumstances (Frankl, 1946/2006). Breitbart and Poppito (2014) translate Frankl's categories of values into concrete sources of meaning that are systematically explored over the course of the intervention: historical sources of

meaning in the sense of one's own life legacy, attitudinal sources of meaning in dealing with the limitations of life, creative sources of meaning in the sense of responsibility and commitment, and experiential sources of meaning in love, beauty, and humor.

In addition, MCP can be understood within the framework of Park's (2010) meaning-making model. A life-limiting diagnosis can create a profound discrepancy between global life goals and the current situation. MCP can be understood as structured support for processes of meaning making that do not deny this discrepancy but embed it in a more comprehensive context of meaning.

A further basic idea is the distinction between what cannot be changed in the face of the illness and what remains open to shaping. The intervention invites participants to perceive both: to acknowledge the limits without losing sight of the remaining possibilities. This reflects Frankl's idea that meaning can be realized not only in doing and experiencing but also in one's stance toward the unalterable.

3 Structure of the Intervention

The group format, Meaning-Centered Group Psychotherapy (MCGP), comprises eight weekly sessions, each devoted to a thematic focus (Breitbart & Poppito, 2014). The sessions combine didactic elements, in which concepts of logotherapy are conveyed, with experiential exercises and group discussion. The program begins with an exploration of the concept of meaning and of the question of how the illness has changed the experience of meaning. Sessions on the various sources of meaning follow, with work on one's own legacy playing a particularly important role. The program concludes with reflection on the shared process and a look at hopes for the remaining time.

For patients for whom participation in a group is not possible, an individual format, Individual Meaning-Centered Psychotherapy, was developed, which provides for a shorter sequence of sessions and can be adapted more flexibly to the course of the illness (Breitbart et al., 2012). Manualization facilitates the training of practitioners and the conduct of controlled studies but at the same time requires a high degree of clinical judgment in order to adapt the structure to the individual needs of seriously ill people.

A characteristic element of the intervention is work on a legacy project. Participants are invited to develop a project that expresses their life and values and points beyond their own lifetime – for instance, recording memories, reconciling with a close person, or a creative project. This element connects the creative values described by Frankl with the question of what remains of a life (Breitbart & Poppito, 2014).

4 State of the Evidence

The first randomized pilot study compared MCGP with supportive group psychotherapy in patients with advanced cancer (Breitbart et al., 2010). The results indicated advantages of the meaning-centered condition with regard to spiritual well-being and experienced meaning; in addition, there were indications of a decrease in hopelessness and desire for death. A pilot study of the individual format likewise showed favorable results compared with a control condition (Breitbart et al., 2012).

A larger randomized trial with active supportive group psychotherapy as the comparison condition essentially confirmed this direction of findings (Breitbart et al., 2015). The meaning-centered group showed advantages with regard to spiritual well-being and quality of life, as well as indications of a reduction in depressiveness, hopelessness, and the desire for hastened death. It is noteworthy that the differences emerged relative to an active control condition that itself contained nonspecific factors such as group cohesion and emotional support. Meaning-centered interventions of this kind were also included in the meta-analysis of existential therapies by Vos et al. (2015) and contributed to the overall favorable picture of structured meaning interventions.

At the same time, the limitations of these findings must be borne in mind. The studies were predominantly conducted at a specialized oncology center in the United States. In a seriously ill population, study dropouts due to disease progression or death are unavoidable, which complicates the interpretation of longitudinal data. Finally, it remains open to what extent the results can be transferred to other diagnostic groups, health care systems, and cultural contexts.

A further aspect concerns the choice of outcomes. In addition to symptom measures, the studies used, in particular, instruments on spiritual well-being and quality of life. This choice corresponds to the aim of the intervention but must be taken into account in comparisons with other psychotherapeutic procedures that aim primarily at symptom reduction. The question of which components of the intervention are responsible for the observed changes has also not yet been conclusively answered.

5 Distinction from Related Interventions

Among the existentially oriented interventions in palliative care, Dignity Therapy has become particularly well known alongside MCP (Chochinov et al., 2005). It aims at strengthening the sense of dignity at the end of life and works with a structured interview from which a written document emerges that can be passed on to relatives. Both approaches share an emphasis on legacy and life review. MCP, however, is more psychoeducational in design, comprises several sessions, and pursues the goal of strengthening the experience of meaning with regard to the remaining lifetime as well, whereas Dignity Therapy is more compact and geared toward the creation of a generativity document.

Common to both interventions is that they understand existential themes not as a mere side effect of mental disorders but as an independent subject of palliative care. In this, they differ from a purely psychopathological approach that views existential distress primarily from the standpoint of comorbid depression or anxiety. In practice, the choice between the approaches may depend on the remaining lifetime, physical capacity, and the preferences of the patients.

6 Implications for Care in the German-Speaking Countries

Several questions arise for implementation in the German-speaking countries. First, careful linguistic and cultural adaptation is required that takes religious and ideological diversity into account and does not implicitly presuppose a particular conception of meaning. Second, delivery places high demands on practitioners: in addition to psychotherapeutic competence, familiarity with existential questions, experience in accompanying the dying, and the capacity for self-reflection in dealing with one's own finitude are indispensable. Third, the introduction should be accompanied by an evaluation that captures not only symptom changes but also the experience of meaning, spiritual well-being, and the acceptance of the intervention by patients and relatives.

Overall, MCP can serve as an example of how basic logotherapeutic ideas can be translated into a precisely described, testable intervention. The available findings are encouraging but do not justify generalization beyond the contexts studied. Further independent replications in different health care systems are desirable.

Relatives and bereaved persons also come into consideration as target groups for meaning-centered interventions. Transfer to these groups is conceptually obvious, since they, too, are confronted with loss, farewell, and questions about the meaning of what has happened; the evidence on this, however, is still limited and requires further research.

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